

DETAILED PROJECT REPORT

Fungi Labs Multi-Speciality Hospital

Submitted in support of a term loan facility of Rs 77.54 Crore

Particulars	Detail
Borrower	Fungi Labs Multi-Speciality Hospital Pvt. Ltd.
Project	100-bed greenfield multi-speciality hospital
Location	Nashik, Maharashtra
Cost of project	Rs 110.78 Cr
Term loan sought	Rs 77.54 Cr (70% of cost of project)
Promoter contribution	Rs 33.23 Cr equity + Rs 17.80 Cr cash-flow support
Total promoter stake in funding	Rs 51.03 Cr — 39.7% of total funding
Repayment	28 quarterly instalments after a 36-month principal moratorium from commissioning
Door-to-door tenor	Approximately 11.5 years
Implementation period	18 months from financial closure
Document	Detailed Project Report — 2 of 4 in the appraisal set
Date	8 August 2026

Basis of preparation and limitations

Fungi Labs is a fictitious entity. This Detailed Project Report is an illustrative case study prepared to demonstrate the structure and content of a bankable DPR. It is not a real financing proposal and must not be relied upon as one.

A DPR submitted to a lender in practice would additionally carry: audited financial statements and net-worth certificates for the promoters, a title search and valuation report on the land, a chartered engineer's cost estimate, quotations for major equipment, draft construction and equipment contracts, statutory approvals obtained to date, and a credit information report on each promoter and guarantor. None of that supporting documentation exists here.

Every figure in this report is generated from the accompanying financial model.

Contents

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1. The proposal in brief

Particulars	Details
Nature of the proposal	Term loan for a greenfield 100-bed multi-speciality hospital
Cost of project	Rs 11,078 Lakh (Rs 110.78 Cr)
Term loan requested	Rs 7,754 Lakh (Rs 77.54 Cr)
Promoter equity in cost of project	Rs 3,323 Lakh (Rs 33.23 Cr)
Additional promoter support facility	Rs 1,780 Lakh (Rs 17.80 Cr) — outside the cost of project
Debt : equity on cost of project	70 : 30
Debt as % of total funding requirement	60.3%
Interest rate assumed	10.75% p.a., floating, linked to the lender's benchmark
Moratorium on principal	36 months from commercial operation date
Repayment	28 quarterly instalments on a ballooning ladder
Working capital facility sought	Rs 900 Lakh cash credit against receivables and stock
Minimum DSCR over the repayment period	1.46x
Average DSCR over the repayment period	1.66x
Project IRR	22.13%
Security offered	First and exclusive charge on all project fixed assets and land; hypothecation of current assets; personal guarantees of the promoters

1.1 Purpose of the facility

The facility will part-finance the construction and commissioning of a 100-bed multi-speciality hospital on a 2-acre freehold site at Nashik, Maharashtra. The facility will comprise 1,05,000 sq ft of built-up area, 33 critical-care beds, four operation theatres, a single-plane cardiac catheterisation laboratory, a 128-slice CT scanner, a 10-station dialysis unit and a full clinical laboratory. Commercial operations are planned 18 months after financial closure.

2. Promoters and management

The lender's first question about a greenfield hospital is not about the building. It is whether the people proposing it can fill it. Hospital projects fail on clinical credibility and management depth far more often than on construction.

2.1 Promoter group

Particulars	Details
Constitution	Private limited company incorporated under the Companies Act 2013
Promoter group	To be specified — a healthcare-experienced promoter group is assumed for this case study
Contribution to the project	Rs 33.23 Cr equity, of which Rs 11.00 Cr is the land contributed in kind at cost
Additional commitment	Rs 17.80 Cr cash-flow support facility, subordinated and non-withdrawable
Total exposure	Rs 51.03 Cr — 39.7% of total funding
Guarantees	Unconditional and irrevocable personal guarantees of the principal promoters

In an actual submission this section would carry: promoter biographies with verifiable clinical or healthcare-operating track record; audited net-worth statements; income-tax returns for three years; a shareholding pattern; details of other business interests and any group-company borrowings; and CIBIL / credit-bureau reports. A lender will not appraise a greenfield hospital without them.

2.2 Management structure to be appointed

Position	Appointment by	Responsibility
Chief Executive Officer	Month 12 of construction	Overall operations, commissioning readiness, payer empanelment
Medical Director	Month 12 of construction	Clinical governance, consultant recruitment and retention, credentialing
Chief Financial Officer	At financial closure	Drawdown management, lender reporting, covenant compliance, revenue cycle
Nursing Superintendent	Month 14	Nursing recruitment, training, critical-care competency
Quality and NABH Coordinator	Month 14	Accreditation roadmap, protocols, clinical audit
Projects Head	At financial closure	Contractor and equipment vendor management, schedule and cost control

Appointing the Chief Executive Officer and Medical Director no later than month twelve is treated in this report as a condition rather than an aspiration. Consultant recruitment, payer empanelment and NABH preparation all depend on someone credible being in post, and each of those has a lead time measured in quarters. A hospital that appoints its leadership at commissioning has already lost its first two quarters of revenue.

3. Project description

3.1 Facility configuration

Component	Specification
Land	2 acres freehold, with floor-space-index headroom for a Phase 2 vertical expansion
Built-up area	1,05,000 sq ft — approximately 1050 sq ft per bed
Operational beds	100, of which 33 are critical care (33%)
Operation theatres	4 — three major modular, one minor / emergency
Cardiac catheterisation laboratory	1 single-plane, with adjacent 6-bed coronary care unit
Imaging	128-slice CT, digital radiography, C-arm, ultrasound and echocardiography. MRI 1.5T deferred to Year 3
Dialysis	10 stations operating three shifts a day, with dedicated RO plant
Clinical laboratory	Fully automated biochemistry, haematology and microbiology; blood storage centre
Support infrastructure	CSSD, medical gas manifold, DG backup, sewage treatment plant, biomedical waste handling, laundry, kitchen

3.2 Bed configuration

Bed category	Beds	% of total
General ward (multi-bedded)	34	34%
Semi-private (twin sharing)	18	18%
Private / deluxe room	12	12%
ICU - medical & surgical	14	14%
CCU - coronary care	6	6%
NICU / PICU	8	8%
HDU / step-down	5	5%
Isolation / negative pressure	3	3%
Total	100	100%

3.3 Clinical specialties

Specialty	Role in the case mix
General medicine	Base occupancy and emergency intake
General and laparoscopic surgery	Plannable volume, moderate ARPOB
Orthopaedics including joint replacement	High ARPOB, plannable, strong insurance and scheme coverage
Cardiology with interventional capability	Primary differentiator; anchors the tertiary-lite positioning

Specialty	Role in the case mix
Obstetrics, gynaecology and neonatology	Recurring family relationships; Level-III NICU capability
Paediatrics	Supports the obstetric programme
Nephrology and dialysis	Recurring, predictable revenue; high scheme coverage
Critical care	Enables higher acuity across every other specialty
Supporting: ENT, ophthalmology, gastroenterology, pulmonology, physiotherapy	Outpatient depth and referral capture

4. Cost of project

Cost head	Rs Lakh	Rs Cr	% of total
Land (freehold)	1,100	11.00	9.9%
Site development & boundary	150	1.50	1.4%
Civil construction	2,835	28.35	25.6%
MEP & building services	998	9.98	9.0%
Interiors & finishes	546	5.46	4.9%
Medical equipment	2,650	26.50	23.9%
Non-medical equipment & utilities	620	6.20	5.6%
Furniture & fixtures	300	3.00	2.7%
IT, HIS & PACS	185	1.85	1.7%
Pre-operative expenses	450	4.50	4.1%
Contingency	331	3.31	3.0%
Interest during construction (IDC)	563	5.63	5.1%
Margin money for working capital	350	3.50	3.2%
Total cost of project	11,078	110.78	100.0%

4.1 Basis of the estimates

Cost head	Basis of estimate
Land	2 acres at Rs 550 Lakh per acre, in the designated institutional zone
Site development	Boundary wall, approach road, levelling, external services — lump sum
Civil construction	1,05,000 sq ft at Rs 2,700 per sq ft, tier-2 rate for hospital-grade structure and finishes
MEP and building services	Rs 950 per sq ft — HVAC, electrical, plumbing, medical gas, fire detection and suppression
Interiors and finishes	Rs 520 per sq ft — antimicrobial flooring, wall protection, modular partitions
Medical equipment	Item-wise schedule at Section 4.2, based on indicative vendor pricing
Non-medical equipment	DG sets, lifts, CSSD, laundry, kitchen, sewage treatment plant
Furniture and fixtures	Beds, cots, cabinets, workstations, waiting-area seating
IT, HIS and PACS	Hospital information system licence, servers, network, PACS, CCTV, access control
Pre-operative expenses	Statutory fees, consultancy, recruitment, pre-launch marketing, two-week trial run
Contingency	4% on hard cost excluding land and pre-operative expenses
Interest during construction	Solved in closed form — see Section 4.3
Margin money for working capital	Promoter margin on the cash credit facility

4.2 Medical equipment schedule

Equipment	Qty	Rs Lakh	Note
Cardiac catheterisation laboratory (single plane)	1	650	Anchor of the cardiology programme
CT scanner — 128 slice	1	420	Emergency and elective imaging
Operation theatre equipment — 4 theatres	4	420	Tables, lights, anaesthesia workstations, electrosurgery
ICU / CCU / NICU equipment	33	380	Ventilators, multi-para monitors, infusion pumps, warmers
Laparoscopy and endoscopy towers	3	175	
Ultrasound, echocardiography, colour Doppler	4	130	
Digital radiography and C-arm	3	115	
Clinical laboratory analysers	1	160	Biochemistry, haematology, microbiology
Dialysis machines and RO plant	10	85	
CSSD and OT support instruments	1	65	
Physiotherapy, audiometry, ophthalmology, ENT	1	50	
Total medical equipment		2,650	As included in the cost of project
Deferred to Year 3 — MRI 1.5T	1	550	Funded from internal accruals, not from the term loan

Deferring the MRI is a deliberate capital-discipline decision rather than an omission. At Year-1 volumes the machine would run at low utilisation while carrying full depreciation and financing cost. Installing it in Year 3, funded from internal accruals once inpatient volumes justify it, removes Rs 5.5 Cr from the amount to be borrowed and improves early-year debt service. The model treats it as expansion capex in operating Year 3.

4.3 Interest during construction

IDC has been computed in closed form rather than by iteration, which avoids a circular reference and makes the calculation auditable. Where d is the debt proportion, r the interest rate, m the construction period in months and f the average drawdown factor, $k = d \times r \times (m \div 12) \times f$, and $IDC = k \times \text{HardCost} \div (1 - k)$.

Component	Value
Hard cost before IDC	Rs 10,515 Lakh
Debt proportion (d)	70.0%
Interest rate (r)	10.75%
Construction period (m)	18 months
Average drawdown factor (f)	0.45 — average debt outstanding over construction on an S-curve drawdown
k	0.05079
Interest during construction	Rs 563 Lakh

4.4 Benchmark comparison

Measure	This project	Reference range
Cost per bed, all-in	Rs 110.8 Lakh	Rs 30–150 Lakh per bed across listed Indian operators
Cost per bed, excluding land	Rs 99.8 Lakh	Rs 70–100 Lakh per bed for a metro build
Built-up area per bed	1050 sq ft	900–1,200 sq ft, NABH planning norms
Civil rate per sq ft	Rs 2,700	Rs 2,400–3,200, tier-2 hospital-grade
Equipment as % of cost of project	29.5%	25–35% typical for a tertiary-lite facility

Cost per bed excluding land sits marginally above the metro reference because the facility carries a cath lab, CT, four theatres and 33 critical-care beds. The equipment share is at the lower end because the MRI has been deferred.

5. Means of finance

Source	Rs Lakh	Rs Cr	% of cost of project
Term loan from bank / financial institution	7,754	77.54	70.0%
Promoter equity — cash	2,223	22.23	20.1%
Promoter equity — land contributed in kind at cost	1,100	11.00	9.9%
Total means of finance — cost of project	11,078	110.78	100.0%

5.1 Additional promoter commitment outside the cost of project

Why a separate facility is required, and why the bank should insist on it

A greenfield hospital loses money for roughly two years. This project records an EBITDA loss of Rs 3.86 Cr in Year 1 and turns EBITDA-positive only in Year 2; profit after tax turns positive in Year 4. Cumulative losses across the first three years are approximately Rs 30.09 Cr.

Banks do not lend against operating losses, and they should not. The promoter must therefore commit a separate facility to fund the start-up deficit and to hold a debt service reserve. This report proposes Rs 17.80 Cr, sized so that the closing cash balance never falls below Rs 150 Lakh in the base case.

It is proposed as an unsecured, subordinated promoter loan with a non-withdrawal undertaking for the tenor of the term loan, so that it counts as quasi-equity for covenant purposes. Documenting it as a condition precedent, rather than relying on the promoter's willingness when the cash runs short, is in the lender's direct interest.

Source	Rs Lakh	Rs Cr
Cost of project — term loan	7,754	77.54
Cost of project — promoter equity	3,323	33.23
Promoter support facility (subordinated)	1,780	17.80
Total funding requirement	12,858	128.58
Term debt as % of total funding	60.3%	
Total promoter contribution	5,103	51.03
Promoter share of total funding	39.7%	

Both views of gearing are presented deliberately. On the cost of project the structure is 70:30, which is what a sanction letter will record. On total funding — the money that actually has to be found before the hospital is self-sustaining — it is closer to 60%:40%. The second number is the one that describes the promoter's real exposure, and a lender assessing commitment should look at it.

5.2 Proposed drawdown schedule

Construction quarter	Drawdown % of sanction	Rs Lakh	Milestone
Q1	6%	465	Mobilisation, foundation
Q2	12%	931	Substructure complete
Q3	20%	1,551	Superstructure to roof level

Construction quarter	Drawdown % of sanction	Rs Lakh	Milestone
Q4	24%	1,861	MEP first fix, envelope closed
Q5	22%	1,706	Equipment advances, interiors
Q6	16%	1,241	Equipment installation, commissioning, licences
Total	100%	7,754	Commercial operation date at end of Q6

Drawdown follows an S-curve, which is why the average debt outstanding during construction is taken at 45% of the final sanction for the purpose of computing interest during construction. Each tranche is proposed to be released against certification by the lender's engineer, with the promoter contribution brought in ahead of or pari passu with each debt tranche.

6. Implementation schedule

Activity	Start	Finish	Duration	Dependency
Financial closure and documentation	M0	M1	1 month	—
Land conveyance and title perfection	M0	M1	1 month	Precedes plan sanction
Soil investigation and detailed design	M0	M3	3 months	—
Municipal plan sanction	M1	M3	2 months	Title, design
Foundation and substructure	M3	M6	3 months	Plan sanction
Superstructure — RCC frame and slabs	M6	M10	4 months	Substructure
MEP first fix	M8	M13	5 months	Runs in parallel from M8
Interiors and finishes	M11	M16	5 months	MEP first fix
Equipment procurement and order placement	M6	M12	6 months	Design freeze
Equipment installation and calibration	M13	M17	4 months	Sealed, dust-free rooms — binding constraint
AERB licensing — cath lab and CT	M14	M17	3 months	Room completion
Fire NOC and statutory clearances	M15	M18	3 months	Blocks opening
Consultant recruitment	M10	M17	7 months	CEO and Medical Director in post
Nursing and paramedical recruitment	M14	M18	4 months	—
Payer empanelment applications	M15	M18+	Ongoing	Revenue-critical
Trial run and soft launch	M17	M18	2 weeks	All licences
Commercial operation date	M18	—	—	Operating Year 1 begins

The real critical path

The structure is not what determines the commissioning date. Three interlocking constraints do.

First, equipment installation requires completed, sealed, dust-free and power-stable rooms — so the cath lab and CT cannot be installed until interiors are finished in those zones, and any slippage in finishes transmits directly to commissioning.

Second, AERB licensing for the cath lab and CT cannot even be applied for until the rooms are physically complete, and it takes weeks to months. Third, an interventional cardiologist recruited in month twelve will not wait indefinitely for a working laboratory.

These three run in series at the end of the programme, which is where schedule risk concentrates. The mitigation is to bring interiors forward in the imaging and cath-lab zones specifically, and to file licence applications the day the rooms are certified rather than after commissioning readiness is declared.

7. Manpower plan

Cadre	Year 1		Mature		Cost per head (Rs Lakh p.a.)					
Full-time consultants (retainer / employed)	16		20		32.0					
Resident medical officers / DNB residents	18		24		9.5					
Nursing staff	105		140		3.9					
Paramedical & technicians	34		46		4.4					
Pharmacy staff	9		12		3.2					
Front office, admissions, billing & TPA	23		31		4.2					
Finance, HR, IT, quality & NABH	11		15		7.0					
In-house support (biomedical, maintenance)	19		21		2.6					
Total headcount	235		309		—					
Staff per bed	2.35		3.09		2.5–3.5 benchmark					
Manpower cost	Y1 FY2029	Y2 FY2030	Y3 FY2031	Y4 FY2032	Y5 FY2033	Y6 FY2034	Y7 FY2035	Y8 FY2036	Y9 FY2037	Y10 FY2038
Total headcount	235	272	292	302	309	309	309	309	309	309
Salary and wage cost (Rs Lakh)	1,494	1,857	2,146	2,407	2,646	2,857	3,086	3,333	3,599	3,887
As % of revenue	49.6%	36.3%	29.8%	27.1%	25.3%	24.6%	24.5%	24.5%	24.5%	24.6%

Consultants are engaged on a hybrid model: a salaried core of 16 rising to 20, supplemented by a visiting fee-for-service pool paid at 11% of inpatient and outpatient revenue. The fixed component secures availability in the specialties that define the case mix; the variable component protects the cost base if volumes disappoint. Nursing is necessarily near-complete from commissioning, because a licensed facility with 33 critical-care beds must be staffed to operate them from day one. That is the largest single contributor to the Year-1 loss and it cannot be phased.

8. Financial projections

8.1 Operating assumptions

Driver	Y1 FY2029	Y2 FY2030	Y3 FY2031	Y4 FY2032	Y5 FY2033	Y6 FY2034	Y7 FY2035	Y8 FY2036	Y9 FY2037	Y10 FY2038
Occupancy	27.0%	43.0%	56.0%	64.0%	70.0%	72.0%	72.0%	72.0%	72.0%	72.0%
Occupied bed-days	9,986	15,768	20,498	23,389	25,491	26,280	26,280	26,280	26,280	26,280
Admissions	2,560	4,043	5,256	5,997	6,536	6,738	6,738	6,738	6,738	6,738
Gross ARPOB (Rs)	26,500	28,487	30,624	32,920	35,389	38,044	40,897	43,964	47,262	50,806
Net realised ARPOB (Rs)	24,088	25,895	27,837	29,925	32,169	34,582	37,175	39,964	42,961	46,183
OPD footfall per day	100	160	220	260	290	310	325	335	345	355

Net ARPOB is gross tariff multiplied by the weighted realisation index of 0.9090, which reflects a payer mix of 40% cash, 35% private insurance and 25% government schemes, settling at 100%, 94% and 72% of list tariff respectively.

8.2 Projected profit and loss

Rs Lakh	Y1 FY2029	Y2 FY2030	Y3 FY2031	Y4 FY2032	Y5 FY2033	Y6 FY2034	Y7 FY2035	Y8 FY2036	Y9 FY2037	Y10 FY2038
Inpatient revenue	2,406	4,083	5,706	6,999	8,201	9,088	9,770	10,503	11,290	12,137
Outpatient revenue	256	433	632	791	935	1,060	1,178	1,287	1,405	1,532
Diagnostics	156	276	411	526	635	719	815	891	991	1,081
Pharmacy	96	169	251	320	385	436	493	538	597	651
Dialysis	60	95	134	162	189	214	232	249	267	286
Other operating income	40	55	70	82	95	105	115	124	133	142
Total revenue	3,013	5,112	7,205	8,880	10,439	11,622	12,603	13,592	14,684	15,831
Consultant payout	293	497	697	857	1,005	1,116	1,204	1,297	1,396	1,504
Consumables	693	1,176	1,657	2,042	2,401	2,673	2,899	3,126	3,377	3,641
Employee benefits	1,494	1,857	2,146	2,407	2,646	2,857	3,086	3,333	3,599	3,887
Other operating expenses	920	1,065	1,295	1,545	1,910	2,057	2,189	2,321	2,453	2,585
Total operating expenditure	3,400	4,595	5,796	6,852	7,962	8,704	9,378	10,077	10,826	11,617
EBITDA	(386)	518	1,409	2,029	2,478	2,918	3,225	3,515	3,858	4,214
EBITDA margin	-13.0%	10.0%	20.0%	23.0%	24.0%	25.0%	26.0%	26.0%	26.0%	27.0%
Depreciation	614	614	655	670	687	618	607	632	660	689
EBIT	(1,000)	(96)	754	1,359	1,791	2,300	2,618	2,883	3,198	3,525
Finance cost	870	891	906	886	825	729	612	479	329	166
Profit before tax	(1,870)	(987)	(152)	473	966	1,571	2,006	2,404	2,870	3,359
Current tax	0	0	0	0	0	0	505	605	722	845

Rs Lakh	Y1 FY2029	Y2 FY2030	Y3 FY2031	Y4 FY2032	Y5 FY2033	Y6 FY2034	Y7 FY2035	Y8 FY2036	Y9 FY2037	Y10 FY2038
Profit after tax	(1,870)	(987)	(152)	473	966	1,571	1,501	1,799	2,147	2,513

8.3 Projected cash flow and debt service

Rs Lakh	Y1 FY2029	Y2 FY2030	Y3 FY2031	Y4 FY2032	Y5 FY2033	Y6 FY2034	Y7 FY2035	Y8 FY2036	Y9 FY2037	Y10 FY2038
EBITDA	(386)	518	1,409	2,029	2,478	2,918	3,225	3,515	3,858	4,214
Less: tax	-0	-0	-0	-0	-0	(0)	(505)	(605)	(722)	(845)
Less: increase in working capital	(286)	(211)	(208)	(167)	(145)	(118)	(97)	(98)	(109)	(114)
Cash available for debt service	(672)	306	1,201	1,862	2,333	2,800	2,623	2,812	3,027	3,254
Term loan interest	870	891	906	886	825	729	612	479	329	166
Term loan principal	0	0	0	465	775	1,008	1,163	1,318	1,473	1,551
Total debt service	834	834	834	1,274	1,517	1,654	1,692	1,714	1,719	1,634
DSCR	—	—	—	1.46x	1.54x	1.69x	1.55x	1.64x	1.76x	1.99x
Closing term loan balance	7,754	7,754	7,754	7,289	6,514	5,506	4,342	3,024	1,551	0
Closing cash balance	826	422	158	535	1,105	1,935	2,531	3,274	4,205	5,426

DSCR is shown only for years in which principal falls due. Years 1 to 3 fall within the 36-month principal moratorium; interest is serviced throughout, funded where necessary from the promoter support facility. Minimum DSCR over the repayment period is 1.46x and the average is 1.66x.

9. Repayment schedule

9.1 Structure

Term	Proposal
Facility	Rupee term loan of Rs 7,754 Lakh (Rs 77.54 Cr)
Purpose	Part-finance the cost of the project as set out in Section 4
Interest rate	10.75% p.a. assumed, floating, linked to the lender's benchmark, reset annually
Drawdown period	18 months from financial closure, in six quarterly tranches against engineer certification
Interest during construction	Capitalised and included in the cost of project
Principal moratorium	36 months from commercial operation date — i.e. the first instalment falls due at the start of operating Year 4
Interest servicing	Monthly throughout, including during construction and the moratorium
Repayment	28 equal quarterly instalments within a ballooning annual ladder aligned to the occupancy ramp
Door-to-door tenor	Approximately 11.5 years — 18 months construction, 36 months moratorium, 7 years repayment
Prepayment	Permitted from internal accruals without penalty after Year 5
Cash sweep	Proposed: 25% of surplus cash above the covenanted minimum applied to prepayment from Year 6

9.2 Annual repayment ladder

Operating year	Financial year	% of sanction	Principal (Rs Lakh)	Interest (Rs Lakh)	Total debt service	Closing balance	DSCR
Year 1	FY2029	—	0	870	834	7,754	Moratorium
Year 2	FY2030	—	0	891	834	7,754	Moratorium
Year 3	FY2031	—	0	906	834	7,754	Moratorium
Year 4	FY2032	6.0%	465	886	1,274	7,289	1.46x
Year 5	FY2033	10.0%	775	825	1,517	6,514	1.54x
Year 6	FY2034	13.0%	1,008	729	1,654	5,506	1.69x
Year 7	FY2035	15.0%	1,163	612	1,692	4,342	1.55x
Year 8	FY2036	17.0%	1,318	479	1,714	3,024	1.64x
Year 9	FY2037	19.0%	1,473	329	1,719	1,551	1.76x
Year 10	FY2038	20.0%	1,551	166	1,634	0	1.99x
Total		100.0%	7,754	6,692	13,706	—	Min 1.46x
Why the ladder is back-ended, and why this is the most important term in the sanction A greenfield hospital does not reach cash break-even until roughly Year 3. In this project the cash break-even occupancy in the stabilised years is about 61%, and occupancy only crosses that level during Year 3.							

Operating year	Financial year	% of sanction	Principal (Rs Lakh)	Interest (Rs Lakh)	Total debt service	Closing balance	DSCR
If principal repayment begins in operating Year 2, the minimum DSCR falls to 0.27x and the covenant is breached in two years — the project is not bankable. Deferring the first instalment to Year 4 lifts the minimum DSCR to 1.46x with no breach in any tested year. The gearing is identical in both cases. Only the timing changes. The ladder then rises from 6% of the sanction in Year 4 to 20% in Year 10, tracking the cash-flow ramp rather than fighting it. That is why this report asks the lender to negotiate on structure rather than on price: the moratorium is worth several multiples of what a pricing concession is worth, and it costs the lender nothing in yield.							

9.3 Quarterly instalment profile

The annual ladder above translates into 28 equal quarterly instalments within each year, beginning at quarter 19 from financial closure. The workbook accompanying this report carries the full 46-quarter schedule — six construction quarters, twelve moratorium quarters and twenty-eight repayment quarters — with opening balance, drawdown, principal, interest accrued, total debt service and closing balance for every quarter, together with a check that total principal repaid equals the sanction.

Summary of the quarterly schedule	Value
Total quarters, financial closure to final instalment	46
Construction quarters (drawdown)	6
Moratorium quarters (interest only)	12
Repayment quarters (principal and interest)	28
Total interest payable over the life of the facility	Rs 6,489 Lakh
Total interest as % of sanction	83.7%

10. Security and covenants

10.1 Security package offered

Security	Particulars	Indicative value (Rs Lakh)
First and exclusive charge on land	2 acres freehold, registered mortgage	1,100
First charge on building and civil works	Registered mortgage	4,379
First charge on plant, machinery and medical equipment	Hypothecation, with vendor invoices endorsed	3,270
Hypothecation of current assets	Receivables and stock, ranking second to the working capital lender	1,016
Personal guarantees	Unconditional and irrevocable, principal promoters	Per net-worth statements
Subordination of promoter support facility	Non-withdrawable for the tenor of the term loan	1,780
Indicative fixed-asset security cover	Fixed assets ÷ term loan sanctioned	1.13x

Security cover on fixed assets is computed on cost, not on realisable value. A lender will apply a haircut, and should: specialised medical equipment has a thin secondary market, and a partly built hospital has very little standalone value. Realistic distress recovery rests on the land and the structure, and on the enterprise being sold as a going concern rather than broken up.

10.2 Covenants proposed

Covenant	Proposed threshold	Modelled	Status
Minimum DSCR, tested annually from Year 4	≥ 1.25x	1.46x	Complies
Average DSCR over the repayment period	≥ 1.40x	1.66x	Complies
Total debt to EBITDA, from Year 6	≤ 3.50x	1.47x	Complies
Current ratio, from Year 4	≥ 1.10x	1.52x	Complies
Promoter contribution to total funding	≥ 30%	39.7%	Complies
Minimum cash balance maintained	≥ Rs 150 Lakh	Rs 158 L	Complies
Total outside liabilities to tangible net worth, Year 5 onward	≤ 2.50x	2.34x	Complies
Total outside liabilities to tangible net worth, Years 1–4	Holiday requested	Peak 4.45x in Year 3	Exception sought

Disclosed exception — leverage on a net-worth basis during the ramp

This report discloses one covenant that cannot be met as conventionally drafted, because it is better raised now than discovered at the first testing date.

Accumulated start-up losses of roughly Rs 30.09 Cr over Years 1 to 3 erode tangible net worth faster than the term loan amortises. Total outside liabilities to tangible net worth therefore peaks at about 4.45x in Year 3 before falling below 0.2x by Year 10. Every greenfield hospital does this. The ratio is simply the wrong instrument during a ramp — it measures the balance sheet at the precise moment the balance sheet is least informative.

Two remedies are requested together. First, a covenant holiday on this ratio until the start of Year 5, with the DSCR and minimum-cash covenants carrying the credit discipline in the interim. Second, recognition of the promoter support facility as quasi-equity in the computation of tangible net worth, supported by a non-withdrawal undertaking for the tenor of the facility.

Covenant	Proposed threshold	Modelled	Status
Both are standard accommodations for greenfield healthcare and neither weakens the lender's position: DSCR is the covenant that actually detects distress in a project like this, and it is proposed at a conventional 1.25x with no relief sought.			

10.3 Conditions precedent proposed

1. Perfection of title to the land and creation of a registered mortgage in the lender's favour.
2. Municipal plan sanction and Pollution Control Board consent to establish obtained.
3. Promoter equity of Rs 33.23 Cr brought in, with the land contribution valued and recorded.
4. Promoter support facility of Rs 17.80 Cr documented as an unsecured subordinated commitment with a non-withdrawal undertaking, and a standby commitment of Rs 25 Cr evidenced.
5. Written commitments from anchor consultants in interventional cardiology, orthopaedics, obstetrics and critical care.
6. Appointment of the Chief Executive Officer and Medical Director, or an agreed timeline not later than month twelve.
7. Fixed-price construction contract and equipment supply contracts executed, with liquidated damages linked to the commissioning date.
8. Comprehensive insurance assigned to the lender: contractor's all-risk during construction, then fire, special perils, equipment breakdown and professional indemnity.
9. Personal guarantees executed and net-worth statements certified.

10.4 Reporting undertakings proposed

- Monthly during construction: physical and financial progress against schedule, with the lender's engineer's certification for each drawdown.
- Monthly during operations for the first three years: occupancy, ARPOB, admissions, OPD footfall, payer mix, receivable ageing and cash balance.
- Quarterly: unaudited financial statements, covenant computation, and a variance statement against these projections.
- Annually: audited financial statements within six months of the year end, and a reconciliation of actual performance against the projections in this report.
- Immediate notification of any event likely to delay commissioning, any departure of an anchor consultant, or any regulatory action.

11. Risk factors and mitigation

This section is deliberately unflattering. A DPR that presents only the base case invites a lender to discover the downside on its own, which is a worse outcome for the borrower than disclosing it.

Risk	Likelihood	Impact	Mitigation
Slow occupancy ramp — the single largest exposure	Medium	Very high	Anchor consultants committed before closure; empanelment applications filed at the earliest admissible date; board-level occupancy triggers
Failure to recruit or retain anchor consultants	Medium	Very high	Hybrid retainer-plus-payout model; multi-year agreements; succession cover in each anchor specialty
ARPOB below plan — adverse case mix or tariff pressure	Medium	High	Cath lab and joint-replacement programmes prioritised; deliberate cap on scheme share of revenue
Construction delay and cost overrun	Medium	Medium	4% contingency; fixed-price contract with liquidated damages; interiors advanced in imaging zones
Delay in AERB, fire or Clinical Establishments clearances	Medium	High	Applications prepared during construction; dedicated liaison resource; sequencing built into the schedule
Delay in payer empanelment	High	High	Applications prepared during construction; NABH entry-level pursued from Year 1
Working capital stretch from scheme receivables	High	Medium	Rs 900 Lakh cash credit facility; dedicated claims function; 90-day realisation target; scheme share capped
Interest rate increase on a floating facility	Medium	Low	Sensitivity tested to 12.5%; ranks second-last of twelve drivers by impact on returns
Competing tertiary facility entering the catchment	Low–Medium	High	First-mover advantage in interventional cardiology; referral relationships built early
Wage inflation, particularly nursing	High	Medium	Modelled at 8% p.a.; retention programmes; in-house training pipeline
Clinical adverse event or reputational damage	Low	Very high	NABH protocols from Year 1; professional indemnity cover; clinical audit and morbidity review

11.1 Scenario testing

Metric	Base	Optimistic	Pessimistic	Lender stress
Revenue Year 5 (Rs Lakh)	10,439	13,180	7,393	5,755
EBITDA margin Year 5	23.7%	34.6%	2.8%	-17.8%
Minimum DSCR	1.46x	2.52x	0.04x	-1.04x
Covenant breach years	0	0	7	7
Project IRR	22.1%	32.9%	-18.5%	n.m.

The borrower's position on the stress case is that it should be read as an argument for the structure proposed in Section 9, not as a reason to decline. Under the pessimistic and stress scenarios this project does not underperform gently; it fails. That is a property of any highly fixed-cost, highly levered greenfield asset. The mitigations that matter are therefore structural — the moratorium, the subordinated support facility, the standby commitment and the operating triggers — rather than incremental improvements to the operating plan.

The accompanying Viability and Bankability Report quantifies each of those mitigations, including a 10,000-trial simulation showing a 91% probability of positive net present value alongside a 27% probability of failing to cover debt service in at least one year. Both figures are disclosed here deliberately.

12. Conclusion and request

Fungi Labs Multi-Speciality Hospital Pvt. Ltd. requests a rupee term loan of Rs 7,754 Lakh (Rs 77.54 Cr) to part-finance a 100-bed multi-speciality hospital at Nashik at a total cost of project of Rs 110.78 Cr, alongside a working capital facility of Rs 900 Lakh.

Summary of the case for support	Evidence
The asset earns more than its cost of capital	Project IRR 22.13% against a WACC of 12.5%; NPV Rs 123.19 Cr
Debt service is covered with headroom	Minimum DSCR 1.46x, average 1.66x, no breach in any tested year
The required market share is modest	Approximately 20% of the estimated 32,130 annual episodes addressable by organised private care in the primary catchment, by Year 5
Cost is benchmarked	Rs 110.8 Lakh per bed, within the range for listed Indian operators
The promoter is materially committed	Rs 51.03 Cr — 39.7% of total funding, plus personal guarantees
Security cover is adequate on cost	1.13x on fixed assets, before haircut
Risks are disclosed, not concealed	Section 11, and the stress testing and simulation in the Viability Report

The borrower asks the lender to note in particular the two structural requests that make this proposal work: a 36-month principal moratorium from commissioning, and a covenant holiday on total outside liabilities to tangible net worth until Year 5. Neither reduces the lender's yield or weakens its security. Both reflect the cash-flow reality of a greenfield hospital, and the analysis in Section 9 demonstrates that the proposal is not bankable without the first.

This Detailed Project Report is document 2 of 4, accompanied by the Feasibility Study, the Viability and Bankability Report, the Learning Companion, the master financial model workbook and the investor presentation. Fungi Labs is a fictitious entity and this document is an illustrative case study, not a financing proposal.